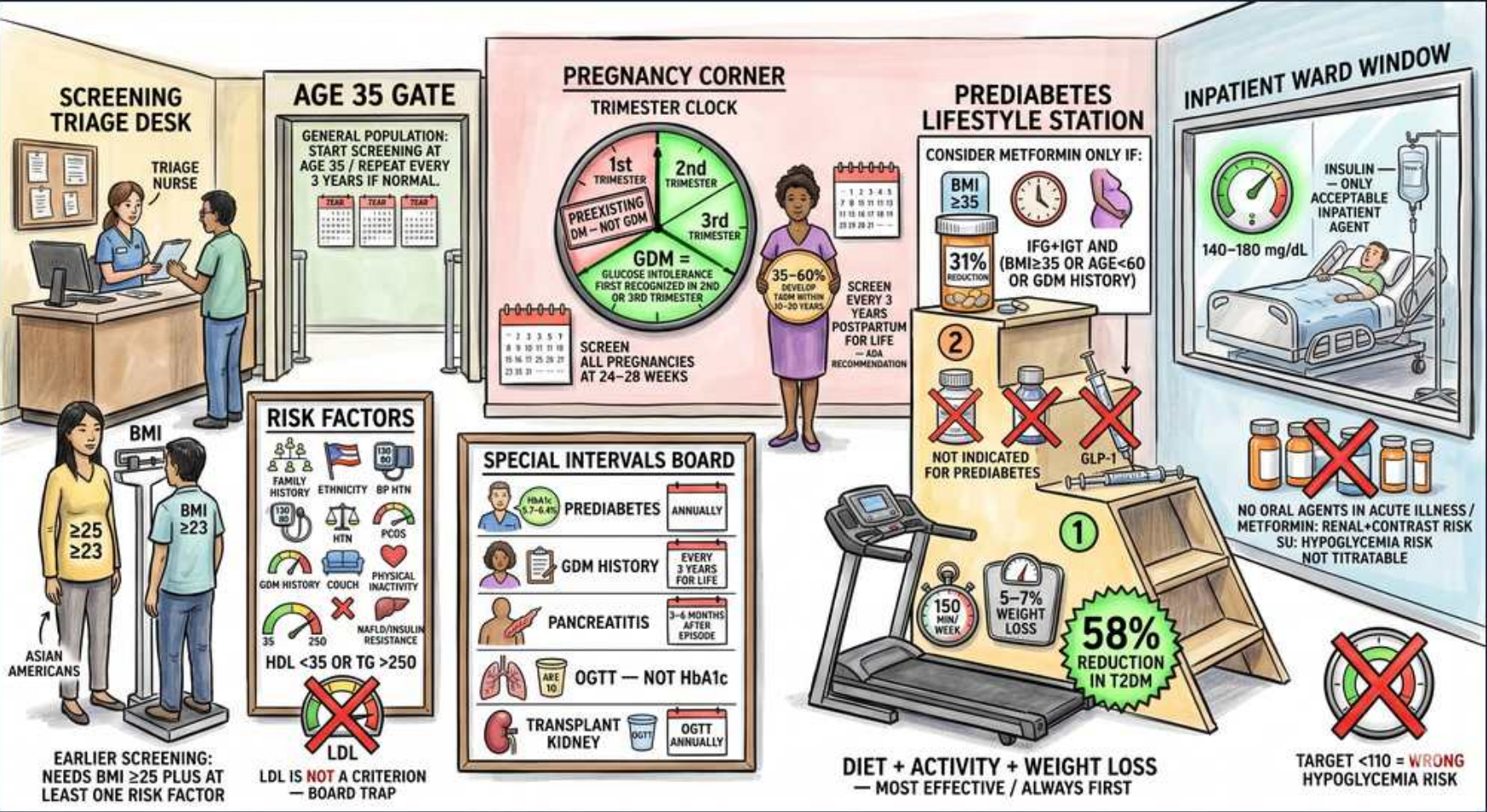


Diabetes — Dx Overview 2: Screening, Prediabetes & Inpatient Targets



1. Age 35 screening gate

WHAT YOU SEE

'Age 35 Gate': general population start screening at age 35, repeat every 3 years if normal.

WHAT IT ENCODES

Universal screening for prediabetes/T2DM in asymptomatic adults begins at AGE 35; if results are normal, repeat at least every 3 years (sooner if risk changes). Screen with FPG, A1c, or 2-h OGTT.

BOARD TRAP

The old cutoff was 45 — the current ADA threshold is 35. The trap is delaying screening to 45, or stretching the normal-result re-screen interval beyond every 3 years.

2. BMI-based earlier screening

WHAT YOU SEE

Body figures: BMI ≥ 25 (or ≥ 23 in Asian Americans) + ≥ 1 risk factor.

WHAT IT ENCODES

Screen EARLIER (at any age, before 35) if OVERWEIGHT — BMI ≥ 25 kg/m² (or ≥ 23 in Asian Americans, who develop insulin resistance at lower BMI) — PLUS at least one additional risk factor.

BOARD TRAP

BMI alone does NOT trigger early screening — it must be paired with ≥ 1 risk factor. And remember the LOWER Asian-American cutoff (23, not 25); the trap is applying the standard 25 threshold to all ethnicities.

3. Risk factors board

WHAT YOU SEE

Risk factors: HTN, family hx, PCOS, prior GDM, low HDL < 35 or TG > 250 , physical inactivity.

WHAT IT ENCODES

Risk factors that lower the screening-age threshold (when paired with BMI $\geq 25/\geq 23$): hypertension ($\geq 130/80$ or on treatment), first-degree family history, PCOS, prior GDM, dyslipidemia (HDL < 35 mg/dL or TG > 250 mg/dL), physical inactivity, high-risk ethnicity, history of ASCVD, and acanthosis nigricans.

BOARD TRAP

The lipid risk factor is LOW HDL or HIGH triglycerides — the trap is listing high LDL as a risk factor (it is not; see next element).

4. LDL is NOT a criterion

WHAT YOU SEE

LDL with red X — 'not a criterion, board trap.'

WHAT IT ENCODES

The dyslipidemia that triggers earlier diabetes screening is LOW HDL (< 35 mg/dL) or HIGH triglycerides (> 250 mg/dL) — both markers of insulin resistance. LDL level is NOT a diabetes screening criterion.

BOARD TRAP

LDL is the classic distractor — a high LDL does NOT lower the diabetes screening age. The discriminator is that diabetes risk tracks with the insulin-resistance lipid pattern (low HDL/high TG), not LDL.

5. Pregnancy / GDM screening

WHAT YOU SEE

'Pregnancy Corner': trimester clock; GDM glucose intolerance first recognized in pregnancy.

WHAT IT ENCODES

Gestational diabetes (GDM) = glucose intolerance first recognized in pregnancy. Screen ALL pregnancies at 24-28 weeks (one-step 75 g OGTT, or two-step 50 g glucose challenge then 100 g OGTT). High-risk women are screened at the first prenatal visit to catch pre-existing (overt) diabetes. Rescreen postpartum at 4-12 weeks with a 75 g OGTT.

BOARD TRAP

GDM screening uses OGTT/glucose challenge, NOT A1c (A1c is unreliable in pregnancy due to increased RBC turnover). And postpartum re-screening is REQUIRED even when GDM resolves after delivery — the trap is stopping follow-up once the patient delivers.

6. Screen at 24-28 weeks

WHAT YOU SEE

'Screen every pregnancy at 24-28 weeks.'

WHAT IT ENCODES

The standard universal GDM screening window is 24-28 weeks gestation, when placental hormone-driven insulin resistance peaks. Earlier screening (first visit) is reserved for those with risk factors to detect undiagnosed pre-existing diabetes.

BOARD TRAP

Don't conflate first-trimester screening for OVERT (pre-existing) diabetes with the 24-28 week GDM screen — the trap is screening only early and skipping the universal 24-28 week test, or vice versa.

7. Special intervals board

WHAT YOU SEE

Prediabetes -> annually; GDM history -> every 1-3 yrs; pancreatitis; OGTT not A1c; transplant kidney; CF annually.

WHAT IT ENCODES

Higher-frequency or OGTT-based screening situations: prediabetes -> ANNUALLY; prior GDM -> at least every 1-3 years for life; post-pancreatitis; post-transplant and CF -> OGTT (not A1c), annually. These groups are at elevated or A1c-discordant risk.

BOARD TRAP

Prediabetes is rescreened ANNUALLY, not every 3 years — the trap is applying the general-population 3-year interval to a prediabetic, who needs yearly testing because of higher conversion risk.

8. Prediabetes thresholds

WHAT YOU SEE

'Prediabetes Lifestyle Station': IFG/IGT; metformin only if A1c, BMI>35 or age<60 or GDM history.

WHAT IT ENCODES

Prediabetes = impaired fasting glucose (IFG, FPG 100-125 mg/dL), impaired glucose tolerance (IGT, 2-h OGTT 140-199 mg/dL), OR A1c 5.7-6.4%. It carries high risk of progression to T2DM and elevated CV risk. Memorize: FPG 100-125 / 2h 140-199 / A1c 5.7-6.4.

BOARD TRAP

Watch the exact ranges: IFG starts at 100 (not 110), and the prediabetes A1c band is 5.7-6.4% (6.5% is already diabetes). The trap is mis-bounding these — e.g., calling FPG 110 'normal' or A1c 6.4% 'diabetes.'

9. Metformin only if high-risk

WHAT YOU SEE

Pill bottle 'not indicated for prediabetes' generally; GLP-1 crossed out.

WHAT IT ENCODES

Metformin in prediabetes is SELECTIVE — reserved for the highest-risk: BMI ≥35, age <60, prior GDM, or rising A1c despite lifestyle. It is NOT for every prediabetic. GLP-1 RAs are NOT indicated to treat prediabetes per se.

BOARD TRAP

If a drug is used in prediabetes it is METFORMIN, not a GLP-1 RA, sulfonylurea, or insulin — and only in high-risk subgroups. The trap is reflexively starting metformin for all prediabetics or reaching for a GLP-1 RA.

10. Lifestyle first — most effective

WHAT YOU SEE

Treadmill: diet + activity + weight loss; 5-7% weight loss = 58% reduction in T2DM.

WHAT IT ENCODES

Intensive lifestyle modification — diet, ≥150 min/week of activity, and 5-7% body-weight loss — is the FIRST-LINE, most effective intervention, reducing progression to T2DM by ~58% (Diabetes Prevention Program). It outperforms metformin (~31% reduction).

BOARD TRAP

LIFESTYLE (58%) beats METFORMIN (31%) for preventing T2DM — the trap is choosing metformin over lifestyle as the primary prevention strategy; lifestyle is always first-line, with metformin reserved for high-risk patients as an adjunct.

11. Inpatient glucose target

WHAT YOU SEE

'Inpatient Ward Window': insulin-only acceptable agent; target 140-180 mg/dL.

WHAT IT ENCODES

For hospitalized patients (both critically and non-critically ill), the glucose target is 140-180 mg/dL, achieved with INSULIN — the preferred/only reliable inpatient agent. Use basal-bolus (scheduled basal + nutritional + correction) for non-critically ill; IV insulin infusion for ICU/critically ill.

BOARD TRAP

Avoid sliding-scale insulin ALONE as the regimen — the trap is managing inpatient hyperglycemia with reactive sliding scale only; scheduled basal-bolus dosing gives better control and fewer swings.

12. No oral agents inpatient / target <110 wrong

WHAT YOU SEE

Red X: no oral agents in acute illness (metformin renal/contrast risk, SU hypo); target <110 = WRONG.

WHAT IT ENCODES

Hold ORAL agents during acute hospitalization: metformin risks lactic acidosis with AKI/contrast/hypoperfusion and must be held around contrast; sulfonylureas cause unpredictable HYPOGLYCEMIA with NPO status; SGLT2i risk euglycemic DKA. Insulin is the inpatient agent. A target <110 mg/dL is too tight/harmful.

BOARD TRAP

The inpatient target is 140-180, NOT <110 — over-tight glycemic control (NICE-SUGAR) increased hypoglycemia and MORTALITY. The trap is selecting an aggressive <110 (or <140) target, or continuing oral agents (metformin/SU) during acute illness.